

PREDNISOLONE
5mg/5ml SYRUP

COMPOSITION:

Each 5ml contains
Prednisolone Sodium Phosphate 5mg
equivalent to Prednisolone
Sodium Benzoate 0.1% w/v as preservative.

PRESENTATION:

Colourless to pale yellow coloured, lychee flavour syrup.

INDICATIONS:

- Corticosteroid indicated conditions include:
- Allergic disorders: bronchial asthma & allergic skin reactions.
 - Blood disorders.
 - Selected collagen & rheumatic disorders.
 - Connective tissue disorders.
 - Gastro-intestinal disorders: inflammatory bowel disease.
 - And also disorders responsive to adrenocortical hormone therapy.

PHARMACOLOGY:

Mechanism of Action:

Prednisolone is an adrenocorticoid with both glucocorticoid and mineralocorticoid properties. Its glucocorticoid potency is four times the potency of equal weight of hydrocortisone. Its mineralocorticoid properties are rather weak and is only one half the potency of hydrocortisone.

Action:

- Prednisolone acts as follow :
- Controls the rate of protein synthesis.
 - Depresses the migration of polymorphonuclear leukocytes and fibroblasts.
 - Reverses capillary permeability.
 - Causes lysosomal stabilization at the cellular level to prevent or control inflammation.

Pharmacokinetics:

Following oral administration of Axcel Prednisolone 5mg/5ml Syrup, prednisolone is rapidly and well absorbed from the GIT. Peak plasma concentrations are achieved within 1-2 hours. About 50% is bound to plasma proteins. Prednisolone is widely distributed into most body fluids and tissues including muscles, liver, skin, intestine and kidney. Prednisolone is extensively metabolized in the liver into sulfate and glucuronide conjugates which are eliminated in urine. Small amounts of prednisolone are eliminated unchanged in urine. Plasma elimination half life is 2-4 hours. Prednisolone crosses the placenta and appears in breast milk.

ROUTE OF ADMINISTRATION:

For oral administration only.

DOSAGE & ADMINISTRATION:

Dosage of Axcel Prednisolone 5mg/5ml Syrup should be individualized according to the severity of the disease and the response of the patient. For infants and children, the recommended dosage should be governed by the same considerations rather than strict adherence to the ratio indicated by age or body weight. Dosage should be decreased or discontinued gradually when the drug has been administered for more than a few days. If a period of spontaneous remission occurs in chronic conditions, treatment should be discontinued.

Initial dose: Varies from 5ml to 60ml/day of Axcel Prednisolone 5mg/5ml Syrup, depending on the specific disease entity being treated. In situations of less severity, lower doses will generally suffice, while in selected patients, higher initial doses may be required. The elderly may require lower doses.

Adults: 5-60mg daily of Axcel Prednisolone 5mg/5ml Syrup in divided doses or as a single dose on alternate days.

In long term therapy: dosage should be maintained at not more than 7mg whenever possible.

Children: As directed by the physician.

For short treatment: not more than 2 weeks, as it may lead to growth retardation in children.

CONTRAINDICATION:

Osteoporosis, active ulcer, psychoses or severe psychoneuroses. Systemic fungal infections. Acute infections unless effective, specific concurrent chemotherapy can be administered. Patients should be not undergo immunization. Tuberculosis, expect as adjunctive treatment with tuberculoestearic drugs.

WARNING AND PRECAUTION:

Precautions:

- Psychosis, acute glomerulonephritis, amebiasis, cerebral malaria, children < 2 years, elderly, AIDS, tuberculosis, diabetes mellitus, glaucoma, osteoporosis, ulcerative colitis, congestive heart failure, myasthenia gravis, renal disease, esophagitis, ocular herpes simplex, live virus vaccines and hypertension. Corticosteroids may mask signs of infection and new infections may appear during their use.

Warning:

- Observe growth and development of infants and children as they are affected on prolonged therapy of corticosteroids.
- The drug may mask signs of infection and the resistance to infections may be diminished.
- Observe for possible delayed wound healing.

Scleroderma renal crisis:

Caution is required in patients with systemic sclerosis because of an increased incidence of (possibly fatal) scleroderma renal crisis with hypertension and decreased urinary output observed with a daily dose of 15mg or more prednisolone.

Pheochromocytoma crisis, which may be fatal, has been reported after administration of systemic corticosteroids. Corticosteroids should only be administered to patients with suspected or identified pheochromocytoma after an appropriate risk/benefit evaluation.

Pregnancy & Lactation:

Axcel Prednisolone 5mg/5ml Syrup should be used during pregnancy only if the potential benefits justifies the potential risk to the fetus. Prednisolone is excreted in breast milk, so caution should be exercised when Axcel Prednisolone 5mg/5ml Syrup is administered to nursing women as it may suppress babies growth.

SIDE EFFECTS:

Fluid and Electrolyte Disturbances: Congestive heart failure in susceptible patients, fluid retention, hypokalemic alkalosis, potassium loss and sodium retention.

CNS: Vertigo, convulsions, headache and psychic disturbances.

Cardiovascular: Hypertension, thrombophlebitis, thromboembolism, tachycardia and congestive heart failure.

Ophthalmology: Exophthalmos, glaucoma, increased intraocular pressure, blurred vision and cataracts.

GIT: Peptic ulcer with possible perforation and hemorrhage, ulcerative esophagitis, diarrhea, nausea, abdominal distension, increased appetite, pancreatitis and perforation of the small and large bowel particularly in patients with inflammatory bowel disease.

Metabolism: Negative nitrogen balance due to protein catabolism, growth suppression in children and decreased glucose tolerance.

Musculoskeletal: Fracture of long bones, steroid myopathy, tendon rupture, vertebral compression fractures, osteoporosis, aseptic necrosis of femoral and humeral heads, muscle weakness and loss of muscle mass.

Skin: Increased sweating, poor wound healing, ecchymosis, bruising, petechiae, striae, thin fragile skin and suppression of skin test reactions.

Endocrine: Decreased carbohydrate tolerance, development of cushingoid state, hirsutism, increased requirements for insulin or oral hypoglycemic agents in diabetes, manifestations of latent diabetes mellitus, menstrual irregularities, adrenocortical and pituitary unresponsiveness, particularly in times of stress, as in trauma, surgery or illness and suppression of growth in children.

DRUG INTERACTIONS:

- Hepatic microsomal enzyme inducers including barbiturates, phenytoin, ephedrine, carbamazepine and rifampin may decrease the corticosteroids effects.
- Aspirin, indomethacin and other NSAIDs may increase the risk of GI distress, bleeding and ulcers.
- Thiazides and diuretics may increase the risk of hypokalemia due to potassium wasting effect of prednisolone.
- The effects of hypoglycemic agents (including insulin) and antihypertensives are antagonized by corticosteroids.

- There is a potential for an increased risk of digitalis toxicity associated with hypokalemia when corticosteroids and cardiac glycosides are used concomitantly.
- Corticosteroids may alter the response to coumarin.

OVERDOSAGE AND TREATMENT :

Accidental ingestion of large doses rarely occurs, however prolonged use and abuse of the drug causes cushingoid changes, moon face, striae, central obesity, hirsutism, acne, ecchymoses, hypertension, osteoporosis, myopathy, sexual dysfunction, diabetes mellitus, hyperlipidemia, peptic ulcer, GI bleeding, increased susceptibility to infection, electrolyte and fluid imbalance and psychosis. Acute treatment of overdoses necessitates immediate gastric lavage or emesis followed by supportive and symptomatic treatment.

STORAGE:

Keep container well closed. Store below 30°C. Protect from light.

**KEEP OUT OF REACH OF CHILDREN
JAUHI DARI KANAK-KANAK**

PACK QUANTITIES:

Available in PET bottle of 60ml and 100ml, amber glass bottle of 120ml.

Further information can be obtained from pharmacist, physician or the manufacturer.

Product Registration Holder & Manufactured By:

 **Kotra Pharma (M) Sdn. Bhd.**
No.1, 2 & 3, Jalan TTC 12, (09002-V)
Cheng Industrial Estate,
75250 Melaka, Malaysia.

REGN - 300823
Date of revision: 30/08/23

MAL23046021AZ